



NAME AND STRENGTH OF ACTIVE SUBSTANCE

Each tablet contains 8 mg bromhexine hydrochloride

PRODUCT DESCRIPTION

Light yellow, round and convex tablet with “Δ” logo on one side and scored on the other

PHARMACODYNAMICS

Bromhexine is an expectorant / mucolytic agent. The drug is a benzylamine derivative (2-amino-3,5-dibromo-*N*-cyclohexyl-*N*-methylbenzylamine hydrochloride) and also a derivative of vasicine and adhatodic acid, alkaloids obtained from the plant *Adhatoda vasica*. Following oral administration, bromhexine has increased sputum volume and reduced the viscosity of bronchial secretions in chronic bronchitis patients. The drug has been reported to induce hydrolytic depolymerization of mucoprotein fibers and stimulate activity of the ciliated epithelium. An increase in lysosomal activity facilitated by bromhexine has been postulated. Improvements in pulmonary function in bronchitis patients appear secondary to easier expectoration. An effect of bromhexine on increasing sputum concentrations of various antibiotics (e.g. oxytetracycline, erythromycin, ampicillin, amoxicillin) has also been reported. However, some of these effects (exocrine stimulation, increased sputum concentrations) have not been confirmed in some studies.

It has been suggested that a metabolite of bromhexine, ambroxol, may contribute to enhanced secretion from exocrine glands during bromhexine administration.

PHARMACOKINETICS

Absorption

Oral, well absorbed. Rapidly absorbed from the gastrointestinal tract. Peak plasma concentrations occur after about 1 hour following oral administration.

Metabolism

Bromhexine undergoes extensive first-pass metabolism in the liver. Ambroxol is a metabolite of bromhexine.

Distribution

It is widely distributed to body tissues. Bromhexine is highly bound to plasma proteins. Bromhexine crosses the blood-brain barrier and small amounts cross the placenta.

Excretion

Bromhexine is excreted primarily in the urine as metabolites. Only small amounts appear as unchanged drug. About 85-90% of a dose is excreted in the urine mainly as metabolites. Approximately 70% of an oral dose of bromhexine has been recovered in the urine within 24 hours. Other excretion: faeces, 4%.

Elimination

It has a terminal elimination half-life of 13-40 hours.

INDICATION

It is used in the treatment of conditions of the respiratory tract associated with retention of mucoid secretions such as bronchitis and asthma, as an aid to expectoration.

DOSAGE AND ADMINISTRATION

- Adults and children > 12 years: 1 tablet three times daily
- Children between 6-12 years: ½ tablet three times daily
- Children between 2-6 years: ½ tablet two times daily
- It may be necessary to increase the total daily dose up to 48 mg in adults

CONTRAINDICATIONS

Disol should not be used in patients who are hypersensitive to bromhexine or any components of the formulation.

WARNINGS AND PRECAUTIONS

Very rare cases of chronically associated severe skin impairments such as Stevens-Johnson syndrome, toxic epidermal necrolysis (TEN), erythema multiforme (EM) and acute generalized exanthematous pustulosis (AGEP) have been reported. In most cases, these could be explained by the severity of the underlying disease or concomitant administration of another drug. In the early stages of such severe skin reactions, initially only nonspecific flu-like symptoms appear (e.g. fever, arthralgia, runny nose, cough, and sore throat). If skin or mucous membrane damage occurs, seek medical advice immediately and discontinue treatment as a precaution.

INTERACTIONS WITH OTHER MEDICATIONS

Administration of bromhexine together with antibacterial agents may enhanced the antibacterial agent concentration in infected tissues.

PREGNANCY AND LACTATION

It should not be used during first trimester of pregnancy or lactation unless advised by the physician.

UNDESIRABLE EFFECTS

Immune system disorders

Frequency not known: Anaphylactic reactions including anaphylactic shock.

Skin and subcutaneous skin disorders

Frequency not known: Severe skin reactions (including Stevens-Johnson syndrome, toxic epidermal necrolysis (TEN), erythema multiforme (EM) and acute generalized exanthematous pustulosis (AGEP).

OVERDOSE AND TREATMENT

In the event of overdose, perform emesis or gastric lavage with necessary supportive and symptomatic measures.

STORAGE CONDITIONS

Store below 30°C

SHELF LIFE

4 years

DOSAGE FORMS AND PACKAGING AVAILABLE

100 tablets per unit box (each box consists of 10's x 10 blister strips)

DATE OF REVISION OF PI

Date of revision: 23/03/2020

Manufacturer :-

SIAM BHEASACH CO., LTD.

123 Soi Chokechairuammitr, Vibhavadi-Rangsit Road, Chomphon, Chatuchak, Bangkok 10900 and 9 Soi Chokechairuammitr 3, Vibhavadi-Rangsit Road, Dindang, Dindang, Bangkok 10400, Thailand.

Product Registration Holder, Importer :-

SPG PHARMA (MALAYSIA) SDN. BHD.

Suite 1701, Block A, Damansara Intan e-Business Park, No. 1, Jalan SS20/27, 47400 Petaling Jaya, Selangor Darul Ehsan, Malaysia.

DIS-8-02-L-MY-ENG-A0005

Copyright © 2020 Siam Bheasach. All rights reserved.

Drawing No. DIS-8-02-L-ENG-MY-A0005

Sent date 02/04/2020